

Report to	IMEDICAL, 1 Union st , Pyrmont, NSW, 2009	Patient	SCHLEGER, SHANE IMEDICAL C/O IMEDICAL 1 UNION ST PYRMONT NSW 2009		
		Phone	0415994541		
		D.O.B	01/01/1985	Age	40 years
				Sex	M
Ref. by/copy to	IMEDICAL,	Collect date	10/02/2025	Lab ref	25-10639735
		Collect time	11:25 AM	Your ref	
		Reported	11/02/2025	02:38 PM	
Tests requested	GH, SCP, HOR, COR, AND, MBA, LIP, GLU, FBE, PRL, SOM, FE, TFT				
Clinical notes					

SERUM GROWTH HORMONE

Growth Hormone < 1.0 mIU/L (< 3.1)

In children, Growth Hormone should be > 20 mIU/L following any appropriate stimulus (e.g. sleep, exercise, hypoglycaemia, arginine). Random values are usually low. Growth Hormone is not recommended to diagnose Growth Hormone excess or deficiency. IGF-1 is the preferred marker, especially for the investigation of Growth Hormone excess.

SURGERY
USE

Normal

No Action/File

Patient
Notified

Make
Appoint.

Further Tests

Notes
Required

Speak
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On Correct
Treatment

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SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Request Number	10636734	10639735
Date Collected	19 Nov 24	10 Feb 25
Time Collected	10:56	11:25
hsCRP (< 4.91) mg/L	0.79	0.67

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 10mg/L should prompt a search for a source of infection or inflammation.

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SERUM HORMONE PROFILE

Specimen Type: Serum

Request Number	Date Collected	FSH IU/L	LH IU/L	PROG nmol/L	E2 (ATEL) pmol/L	E2 (BECK) pmol/L	LH/FSH Ratio
10636734	19 Nov 24	14	6.1	2	100		
10639735	10 Feb 25	9	3.6	2	77		

Reference Ranges	FSH	LH	PROG	E2 (ATEL)
Males < 60 yrs	<7	<7	0.9-3.9	<150
> 60 yrs	<14	<11	0.9-3.9	<150

PLEASE NOTE:

- 'E2 (ATEL)' - Oestradiol by Siemens Atellica assay
- 'E2 (BECK)' - Oestradiol by Beckman Access assay

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SERUM CORTISOL

Time 11:25
Cortisol 387 nmol/L

AM Reference Interval 120-620 nmol/L

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SERUM ANDROGENS					
Total Testosterone (Siemens)	17.4	nmol/L		(8.3-29)	
Sex Hormone Binding Globulin	37	nmol/L		(11-71)	
DHEAS	6.8	umol/L		(3.0-10)	
Calculated Free Testosterone	337.0	pmol/L		(255.0-725.0)	

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SERUM CHEMISTRY

Request Number 10636734 10639735
Date Collected 19 Nov 24 10 Feb 25
Time Collected 10:56 11:25
Specimen Type: Serum

Haemolysis		Nil	Nil
Icterus		Nil	Nil
Lipaemia		Nil	Nil
Na	(135-145) mmol/L	141	140
K	(3.6-5.4) mmol/L	4.0	4.1
Cl	(95-110) mmol/L	101	103
HCO3	(22-32) mmol/L	26	26
An Gap	(10-20) mmol/L	18	15
Urea	(2.5-7.5) mmol/L	5.7	5.2
Creat	(60-110) umol/L	100	95
eGFR	mL/min/1.73sqM	79	85
Urate	(0.20-0.42) mmol/L	0.43	0.40
Bili	(< 20) umol/L	17	7
AST	(< 40) U/L	22	21
ALT	(< 40) U/L	27	32
GGT	(< 50) U/L	20	15
Alk Phos	(35-110) U/L	62	67
Protein	(60-82) g/L	74	69
Albumin	(38-50) g/L	46	44
Glob	(20-38) g/L	28	25
Ca	(2.10-2.60) mmol/L	2.50	2.37
Corr Ca	(2.10-2.60) mmol/L	2.44	2.35
P04	(0.75-1.50) mmol/L	1.03	1.08
Mg	(0.70-1.10) mmol/L	0.86	0.84

eGFR values between 60 and 89 mL/min/1.73m2 should be interpreted with caution. These results are only consistent with CKD in the presence of other evidence such as microalbuminuria, proteinuria or haematuria.
Ref:Lamb EJ etal in Ann Clin Biochem 2005; 42:321-345.

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LIPID STUDIES

Request Number 10636734 10639735
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Time Collected 10:56 11:25
Specimen Type: Serum

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis	Nil	Nil
Icterus	Nil	Nil
Lipaemia	Nil	Nil

Fasting status		Fasting	Fasting
Chol (3.9-5.2)	mmol/L	4.3	4.2
Trig (0.5-1.7)	mmol/L	1.1	0.8
HDL (1.0-2.0)	mmol/L	1.1	1.0
LDL (1.5-3.4)	mmol/L	2.7	2.8
Non-HDL (< 3.4)	mmol/L	3.2	3.2
Chol/HDL(< 5.0)		3.9	4.2

NVDPa TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL-C	>= 1.0
LDL-C	<2.0
NON HDL-C	<2.5

LDL-C exceeds target for higher risk patients and may be excessive in some individuals.

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Clinical notes					

SERUM/PLASMA GLUCOSE

Request Number	10636734	10639735
Date Collected	19 Nov 24	10 Feb 25
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Fasting status	Fasting	Fasting
Serum (3.4-5.4) mmol/L	5.1	5.2

Normal glucose concentration.

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Normal

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Clinical notes

HAEMATOLOGY

Request Number	10636734	10639735
Date Collected	19 Nov 24	10 Feb 25
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Specimen Type: EDTA		
Hb (130-180) g/L	157	152
Hct (0.40-0.54)	0.45	0.44
RCC (4.5-6.5) x10 ¹² /L	5.2	5.2
MCV (79-99) fL	87	84
MCH (27-34) pg	30	29
MCHC (320-360) g/L	351	348
RDW (10.0-17.0) %	12.1	12.1
WBC (4.0-11.0) x10 ⁹ /L	6.0	6.2
Neut (2.0-7.5) x10 ⁹ /L	2.7	3.0
Lymph(1.0-4.0) x10 ⁹ /L	2.6	2.4
Mono (0.2-1.0) x10 ⁹ /L	0.4	0.4
Eos (< 0.7) x10 ⁹ /L	0.3	0.4
Baso (< 0.2) x10 ⁹ /L	0.1	0.1
Plat (150-400) x10 ⁹ /L	284	258

HAEMATOLOGY: FBC parameters are within reference range.

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Clinical notes					

PROLACTIN

Specimen Type: Serum

Request Number	Date Collected Ref Range	Prolactin mIU/L (40-450)
10636734	19/11/24	267
10639735	10/02/25	172

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Further Tests
Notes Required
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Clinical notes

INSULIN-LIKE GROWTH FACTOR-1 (IGF-1)

Specimen Type: Serum

IGF-1 (Somatomedin C) 20 nmol/L (13-36)

Insulin-like growth factor 1 (IGF1) and insulin-like growth factor-binding protein 3 (IGFBP3) measurements can be used to assess growth hormone (GH) excess or deficiency. For all applications, IGF1 measurement has generally been shown to have superior diagnostic accuracy. In particular, for the diagnosis and follow-up of acromegaly and gigantism, IGFBP3 measurement adds little extra diagnostic information.

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IRON STUDIES

Request Number	10636734	10639735
Date Collected	19 Nov 24	10 Feb 25
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Specimen Type: Serum		
Iron (10-30) umol/L	26	13
T'ferrin(27-46) umol/L	28	26
T. Sat. (13-45) %	47	26
Ferritin(30-300) ug/L	194	155

Transferrin may be decreased by inflammation (acute or chronic), or protein deficiency or loss. The ferritin concentration excludes iron deficiency.

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THYROID PROFILE				
Request Number	10636734	10639735		
Date Collected	19 Nov 24	10 Feb 25		
Time Collected	10:56	11:25		
Specimen Type: Serum				
TSH (0.5-4.0)	mIU/L	1.4	1.6	
FT4 (10-20)	pmol/L	17	14	
FT3 (3.5-6.5)	pmol/L	5.9	5.9	
Result(s) consistent with euthyroidism.				

**SURGERY
USE**

Normal
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Further Tests
Notes Required
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